

The Rise of Anxiety and Depression Among Young Adults in the United States



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Summary+

Anxiety and depression are both severe mental health disorders that have had a serious impact not only on the mental health but on the educational, professional, and other life outcomes of many individuals in the United States. The prevalence of both of these disorders within the US has increased significantly since the mid-twentieth century. One of the most at-risk age populations is young adults, who are typically defined as individuals between 18–30 years of age. While this increase in both anxiety and depression among young adults is a complicated issue with many causes, major contributing factors include greater levels of social media engagement, academic stress, and economic stress. Some also claim that the rise in these disorders is due to an increased amount of awareness; however, research and evidence on this matter is limited. The increased prevalence of these debilitating disorders has hurt the academic and vocational performance of young adults, irreparably damaged intrapersonal relationships, caused many to develop co-occurring substance abuse disorders, and led many to die by suicide. Despite these adverse outcomes, though, several promising interventions are providing hope to depressed and anxious young adults throughout the US. Practices that improve distressed individuals' access to needed services, empower young adults through self-love, increase their access to validated treatments and qualified mental healthcare providers, and focus on building communal and societal support for young adults with mental illness are paving the way to a brighter future for young adults in the US struggling with mental disorders.

Key Takeaways+

- Anxiety and depression is becoming increasingly prevalent among young adults in the US, with both disorders increasing by 63% from 2005 to 2017.
- The expansion of media multitasking and high levels of academic and economic stress have led to increasing rates of anxiety and depression among American young adults.
- Rising levels of depression and anxiety have resulted in decreased performance in academic and professional settings, strained relationships, high levels of substance abuse, and suicide.
- Social support and counseling services have been largely proven to decrease levels of anxiety and depression.
- Suffering individuals need quality care and proven treatment to recover fully.

Key Terms+

Generalized Anxiety Disorder—Perpetual and excessive feelings of anxiety and worry that are difficult to handle and impede everyday living.¹

Major Depressive Disorder—Ongoing feelings of sadness and despair, coupled with a loss of interest in activities one usually enjoys, that can disrupt normal living and lead to various physical and emotional issues.²

Mood Disorders—General distortion of emotional states that interfere with normal functioning and living; one of the many examples of a mood disorder is major depressive disorder.³

Anxiety Disorders—A category of mental health disorders that involve repeated episodes of sudden and intense feelings of anxiety and fear; this encompasses generalized anxiety disorder but would also include

other conditions such as specific phobias, panic disorders, and obsessive-compulsive disorder.⁴

Young Adults—Individuals between ages 18 and 30.

Substance Abuse—The hazardous use of psychoactive substances, including alcohol and illicit drugs.⁵

Media Multitasking—The simultaneous engagement of multiple media sites.⁶

Context

Q: What is anxiety and its symptoms?

A: **Generalized anxiety disorder** (GAD) is defined by the American Psychiatric Association as exhibiting symptoms such as excessive worry and anxiety on most days about upcoming events or activities. An individual with GAD may experience restlessness, fatigue, difficulty concentrating, irritability, muscle tension, and sleep disturbance.⁷ In the United States, individuals who experience excessive worry throughout a period of six months and have at least three of the following symptoms: high levels of nervousness, an increased heart rate, hyperventilation, insomnia, etc. can be



clinically diagnosed with GAD.⁸ For the purposes of this brief, the terms GAD and anxiety are used interchangeably.

Q: What is depression and its symptoms?

A: Symptoms of major depressive disorder (MDD) defined by the American Psychiatric Association include depressed mood (dysphoria), a loss of interest in activities that were pleasurable in the past, an unhealthy change in sleep and eating habits, increase in purposeless physical activity (inability to sit still, pacing, handwringing), feelings of worthlessness or inappropriate guilt, suicidal ideation, and other symptoms.⁹ For the purposes of this brief, the terms MDD and depression are used interchangeably.

Q: What is the diagnosis criteria for mental illness in the United States?

A: The American Psychiatric Association created the *Diagnostic and Statistical Manual for Mental Health Disorders (DSM-5)*, which is the main guide used by health professionals in the United States and other parts of the world to diagnose mental health disorders.¹⁰ Health professionals refer to this manual to diagnose mental illnesses such as anxiety and depression, as well as others.

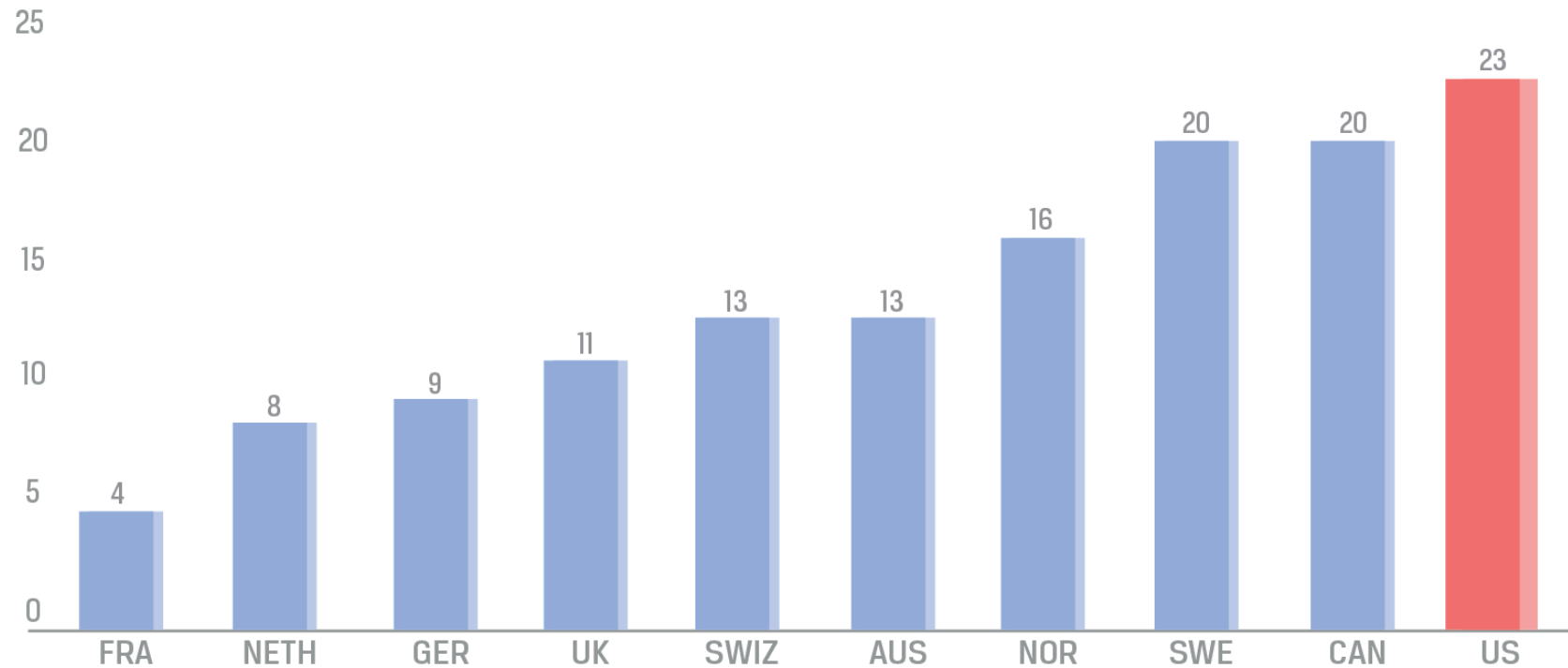
Q: What is the history of anxiety and depression in the United States?

A: Mental illnesses such as anxiety and depression have been documented as early as the second millennium BCE in ancient Mesopotamia but were not declared a legitimate dysfunctions of the mind by doctors until the 1600s.¹¹ Shortly after the end of World War II, anxiety began to rapidly claim the mental state of millions of Americans and depression grew by 300% from 1987 to 1997. Since then, anxiety and stress disorders have continued to increase in frequency of clinical diagnosis, particularly among **young adults** in the US.¹²

Young adults in the US have become increasingly more anxious and depressed since the 1930s.¹³ A 69-year longitudinal study conducted in the US measured the prevalence of multiple mental health disorders such as anxiety, depression, hypomania, and others by using the Minnesota Multiphasic Personality Inventory (MMPI) as criteria. The study determined that, though there was variance in the average MMPI scores of college students year to year, there has been an overall rise in MMPI scores over time.¹⁴ In 2017, individuals between the ages of 18 and 25 had the most reported depressive episodes (13%) compared to individuals in older age groups (7.7% and 4.7%).¹⁵ Additionally, depression and anxiety diagnoses have increased by 63% among young adults from 2005 to 2017.¹⁶ As of 2016, 23% of adult US citizens have reported having a mental health diagnosis, which is a high rate in comparison to other high-income countries such as Canada (20%), New Zealand (13%), the United Kingdom (11%), Germany (9%), and France (4%).¹⁷

Mental Health Diagnoses Among Adults in High-Income Countries

Percent of participants who responded "yes" to the question: "Have you ever been told by a doctor that you have, or have had, depression, anxiety, or other mental health problems?"



Contributing Factors

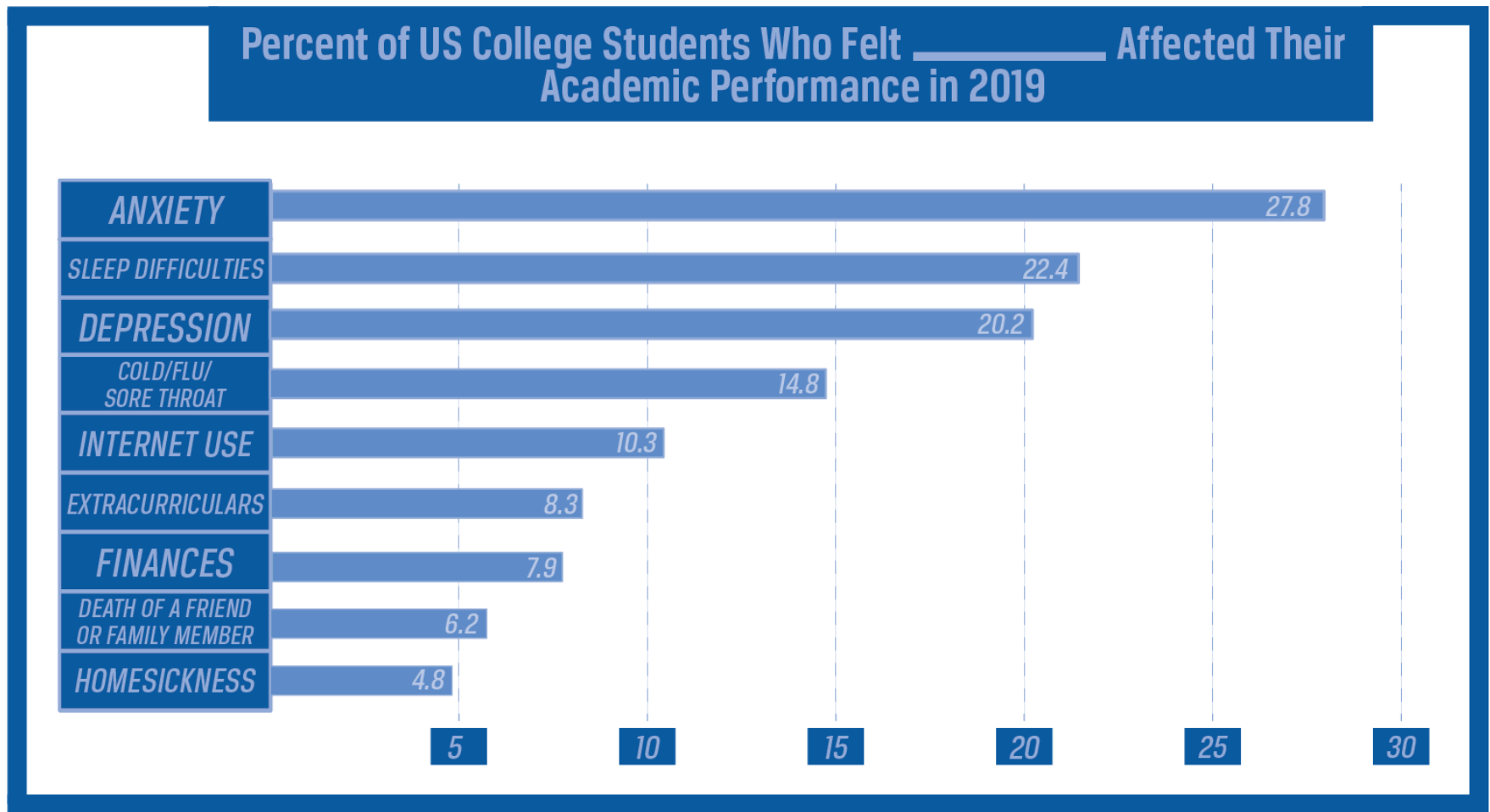
Social Media

Increased social isolation among young adults has been statistically linked to increasing levels of depression and anxiety. As young adults connect more on social media, many feel even more socially isolated because of constant exposure to highly edited, filtered, and unrealistic portrayals of the lives of others. According to an observational study done in 2016, young adults in the higher quartiles of time spent on social media had at least a 15–20% higher chance of having depression compared to those in the lower quartiles of social media use.¹⁸ Furthermore, **young adults** who use 7 or more social media platforms are statistically 3 times more likely to experience increased levels of depression and anxiety than young adults who use 2 or less.¹⁹ Although more research needs to be done, individuals may be up to 46% more likely to have depression if they are using social media more than 60 minutes per day.²⁰



Academic Stress

With 40% of the **young adults** in the US attending 2 or 4 year institutions as either undergraduate or graduate students, academic stress is a significant factor and can explain the rise of anxiety and depression among this cohort.²¹ In 2015, 42.8% of American college students reported above-average stress, while 13.8% reported depression affecting their individual academic performance.²² In 2019, 23.1% of American college students felt hopeless, 22.1% felt overwhelming anxiety, 23.0% felt very lonely, 19.7% felt so depressed that it was difficult to function, and 8.6% seriously considered suicide at least once in the previous 12 months.²³ When compared to well-being before attending college, students' levels of stress, anxiety, and depression all appear to increase as students progress through their undergraduate careers and never return to pre-registration levels throughout the rest of their college experience.²⁴ Additionally, university students in the US report greater amounts of stress by about 20% due to pressure to perform well or the extra time spent on extracurricular involvement and more severe reactions to stress in comparison to their peers internationally.²⁵ For example, heavy workloads and high pressure to perform well have been associated with depression among university students.²⁶ These stressors are highly associated with both anxiety and depression and are also accompanied by a myriad of undesirable physical symptoms such as constant fatigue, which has been reported by 35.2% of college students.²⁷ Additionally, 45.3% of college students in 2019 felt "more than average stress" within the previous 12 months.²⁸



Economic Stress

Another stress factor that has statistically proven to decrease the mental wellness of young adults in America is financial or economic stress. In 2019, 36.9% of American college students reported that finances were "traumatic or very hard to handle."²⁹ College debt, navigation through America's economy, emerging careers, and possibly first-time homeownership can prove to be a mentally strenuous transition for **young adults** recently graduated from college.³⁰ Currently in the United States, there are 44 million student loan borrowers (total sum amount of \$1.5 trillion in debt), and 1 in 15 borrowers from a sample group of this population has considered suicide due to their student loans.³¹ Another 15-year longitudinal study on young adults found that those with student debt reported 11.7% higher symptomology of stress than the adjusted mean of the average citizen, and 13.2% higher symptomology of depression.³² Young adults experiencing financial struggles are 10% more likely to have anxiety and depression and are at a higher risk to report suicidal thoughts.³³

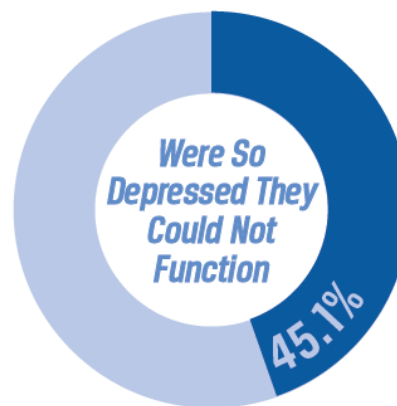


Consequences

Suicide

Widespread anxiety and depression among **young adults** in the US have led to high rates of suicide. Of the more than 47,000 people who died by suicide in 2019, 90% had a diagnosable mental health illness.³⁴ While that figure also includes mood and personality disorders outside the scope of anxiety and depression, suicide is still widely prevalent among young adults diagnosed with both disorders. In 2019, 13.3% of American college students reported seriously considering suicide and 2.0% reported attempting suicide.³⁵ According to the National Epidemiologic Survey on Alcohol and Related Conditions in 2010, 70% of individuals who reported ever attempting suicide had an **anxiety disorder**.³⁶ There was an overall 30% increase in suicides within the US from 2000 to 2016.³⁷ While suicide increased in every age group, young adults are of particular concern due to unique social factors of their age demographic, such as social media use, anxiety, depression and self-inflicted injuries (which was the second leading cause of death for individuals ages 10–24 in 2015³⁸).³⁹ High rates of underdiagnosis and undertreatment of mental health disorders like depression and anxiety put affected individuals at a higher risk for suicide.⁴⁰

Percent of US College Students Who _____ in 2019



Impaired Academic and Professional Achievement

Anxiety and depression can directly impede both academic and professional performance. In college students, anxiety and depression are associated with lower levels of academic achievement, poor occupational preparedness, and decreased future occupational performance.⁴¹ One study found that 92% of students with moderate to severe depression exhibited signs of academic impairment (they were more likely to miss class, have lower grades, and experience significant interpersonal problems in school). Researchers also found that more severe depression is correlated to a higher level of impairment.⁴²

Professional performance can be negatively affected by mood and **anxiety disorders** as well. Many **young adults** with depression report difficulty finding and keeping a job, as depression in the workplace is associated with absenteeism and decreased productivity.⁴³ Research suggests that employees with depression have to face more obstacles in their work due to their mental illness, which can in some cases create deficits in their work output.⁴⁴ In fact, it is estimated that depression impairs work performance 29% more than several chronic diseases do, including arthritis, back problems, diabetes, and high blood pressure.⁴⁵

Strained Interpersonal Relationships

Young adults in the US experiencing depression and anxiety can face significant challenges in their interpersonal relationships,⁴⁶ which can result in compounding feelings of social isolation and loneliness.⁴⁷ Not only are those who experience low-quality social interaction two times more likely to develop depression compared to their more gregarious counterparts,⁴⁸ but also 80% of mental health patients agree that mental health conditions have had an adverse effect on their family relationships.⁴⁹ According to the World Health Organization, one in four families have at least one member of the family that struggles with mental illness and, although it is hard to quantify the stress felt by the rest of the family, a member of the family with a mental illness often has a significant impact on the stress and interpersonal relationships within a family. This may be due to the fact that family members often function as primary sources of emotional support and care for each other.⁵⁰

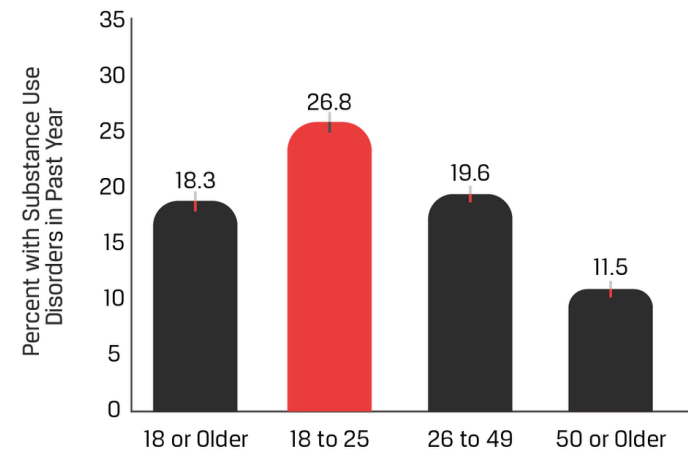
Relationships with romantic partners can be especially difficult to foster and maintain when the individual is dealing with mental health issues. Many depressed young adults have impaired social skills, decreased sexual drive, and doubts about their relationships. They also tend to avoid emotional disclosure, which can lead to conflict and dissatisfaction for both partners.⁵¹ Approximately 75% of people who struggle with depression report having a lack of sex drive, which can prove to be a major roadblock in many intimate relationships.⁵² Panic attacks and episodes of fear and worry also diminish young adults' ability to communicate their needs to their partner, to understand their partner's needs, and to prioritize their needs

over the needs of their partner. It can also foster unnecessary feelings of distrust and suspicion that can, in turn, increase the individual's feelings of anxiety and cause them to feel emotionally isolated from their partner.⁵³

Substance Abuse

Many young adults in the US turn to alcohol or drugs to manage their mental illness or to decrease the negative side-effects of psychiatric medications.⁵⁴ The National Bureau of Economic Business reported that 69% of America's alcohol and 84% of the nation's cocaine is consumed by those who have been diagnosed with a mental illness at some point in their lives.⁵⁵ According to the Anxiety and Depression Association of America, 20% of Americans with an anxiety or mood disorder turn to substance abuse to alleviate their symptoms.⁵⁶ The relationship between mental illness and heavy alcohol use is cyclical, meaning that one can lead to the other.⁵⁷ Because of this cycle, it may be difficult to pinpoint causation in situations of co-occurring mental health and alcohol issues. Many young adults with anxiety and depression have co-occurring substance use disorders (SUD),⁵⁸ which includes addictions to and abuse of substances such as alcohol and drugs that negatively impact an individual's health, home, and social life.⁵⁹ In 2017, 26.8% of young adults in the US who struggled with an SUD also suffered from another mental

Past Year Substance Use Disorders Among Adults with Any Mental Illness in 2017



health disorder as well.⁶⁰ Together, these three disorders (anxiety, depression, and SUD) represent some of the most frequent and burdensome psychiatric problems in the US with 32% of patients with diagnosed mood disorders also exhibiting SUD concurrently.⁶¹ Additionally, 16.5% of individuals who had ever been diagnosed as seriously depressed in their life also had an alcohol use disorder, and 18% of the same group had a drug use disorder.⁶² Young adults between the ages of 18 and 25 have the highest percentage rate of concurrent mental illness and SUD at 26.8%, while other cohorts experience lower rates (ages 26–49: 18.6%, ages 50 or older: 11.5%).⁶³

Practices

Counseling

Mental health counseling is one of the first solutions that people of all ages consider when experiencing mental health illness. Psychotherapy ("talk therapy") as well as antidepressant medications have been proven to be highly efficacious for both anxiety and depression disorders.⁶⁴ Cognitive behavior therapy has shown 46% clinical improvement in treated individuals experiencing anxiety and has been reviewed as one

of the best types of therapy for youth.⁶⁵ Additionally, college students have the opportunity to visit counseling clinics that are on campus and specifically tailored towards student needs (including low to no attendance fees).⁶⁶ It is important for students to see college health nurses, who can assess the coping style of students and help them learn positive coping methods.⁶⁷ From a sample size of more than 85,000 college students from 2007–2020, 50.9% individuals with depression and 55% individuals with anxiety reported seeking therapy and medication.⁶⁸ According to a 2014 National Survey of College Counseling Centers completed by the University of Pittsburgh, 52.45% of colleges surveyed have psychiatric evaluations available, 85.7% believe that there has been an increase of student clients on psychiatric medication prior to seeing a counselor, and 58% report seeing an increase in students with clinical depression while 89% report an increase in **anxiety disorders** in the last five years.⁶⁹ Counseling services made available to **young adults** offer relief from ever-increasing mental stress and disorders and should always be considered by those who suffer from any of these disorders.

Impact

Counseling and different therapy methods can significantly help struggling individuals with anxiety and depression. In a randomized control study, high school and university students were assigned to two different groups, one that received problem-solving therapy (PST) and another that was put in a waiting list (WL) control group. After a 12-month long follow-up period, post-treatment depression and suicide risk significantly decreased in the PST group compared to unchanged results from the WL group. Overall, 77.8% of the PST group compared to 15.8% of the WL group experienced a full or partial recovery.⁷⁰ Another study that was performed in a similar manner with a waiting list (WL) control group and a therapy group (this time, solution-focused brief therapy or SFBT), found that after four and a half months, the SFBT group had significantly improved by 28.2% compared to 11% by the WL control group in their anxiety and depression

symptoms.⁷¹ Additionally, after a 5-week intervention program, **young adults** experienced a 9% decrease in both anxiety and depression whilst participating in either in-person counseling or virtual counseling through Internet services.⁷²

Gaps

Despite the success that counseling can offer, many young adults with anxiety and depression choose not to seek counseling out.⁷³ According to one survey, 42% of young adults weren't able to pay for it, 38.9% thought that their mental health issues would eventually go away, and 32.5% didn't have the time.⁷⁴ Additionally, another review of 144 studies including 90,189 participants found that negative stigma (of receiving counseling) was the fourth most common barrier to those who need counseling services.⁷⁵ Due to this issue of accessibility among **young adults**, health care professionals have begun creating self-care apps and other smartphone apps that are free or low in price and offer mental-health care services. However, although 70% of a survey group of 320 mentally ill patients in America have shown interest in the usage of virtual counseling or smartphone apps offering mental health services, the evidence of their impact and efficacy is limited.⁷⁶

Crisis Lines

Crisis-lines have become widely popular among anxious and depressed populations because they extend opportunities to distressed youth to open up to somebody about their problems through call or text, knowing that their identity will not be disclosed.

2019 Skoll awardee, Crisis Text Line, took the rise of anxiety and depression seriously and started the first and only national, 24/7 crisis-intervention hotline⁷⁷ in order to assist individuals who feel limited in their options of where to go for help. Created in 2013, Crisis Text Line is determined to offer its services to those who suffer through text support, exchange of contact information of those qualified to help that live nearby their clients' area, additional lists of resources that they can turn to, as well as a person to talk to.

Impact

Crisis Text Line has received over 100 million messages, with 600,000 conversations per month and 40 messages exchanged per conversation on average so far.⁷⁸ Seventy-five percent of their texters are under the age of 25.⁷⁹ Sixty-five percent of texters say that they have shared something with Crisis Text Line that they haven't shared with any other human and they also carry out 10 active rescues per day, which is where an emergency has taken place or will take place and text counselors need to call the police and send them to the texter who is in potential harm.

Crisis Text Line claims itself to be the largest mental health data set that has ever been collected, stored, and analyzed. Because they receive messages from those who are experiencing all kinds of distress (anxiety, depression, suicidal ideation, abuse of any type), they have created word banks for each situation that tell them what words are most used when someone has anxiety, depression, or other distress. They provide this information to counselors, schools, families, and other professionals that can then be more prepared if an individual were to start saying a lot of these same words.

Gaps

Although the Crisis Text Line has provided relief to millions and provides the public with transparent data, they don't explicitly provide information on the future progression of their texters' mental health status. It is unknown whether or not specific texters have actually accepted help, reached out to one of the provided resources, and/or if they are very frequent texters and can't seem to move on. It is possible that the texter may develop a bond of trust with one of the counselors at Crisis Text Line, which would hinder further healing and growth of the texter, because they have made friends with someone who is only available virtually. Other researchers have claimed that while crisis lines do appear to be effective in giving aid to those in emergency situations, research is limited, so their actual impact is inconclusive.⁸⁰ Lastly, oftentimes, crisis lines have difficult 10 digit numbers to remember; thus it can be hard to remember to text or call one in the midst of an emergency. Though the federal government is attempting to create a three-digit number (like 911) that could be used for crisis lines, the resource has not yet been provided.⁸¹

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Footnotes

North America

Emma Kauana Osorio & Emily Hyde

Emma is a senior majoring in exercise and wellness with a minor in French. She has a passion for people and helping those who are in need. She is a certified yoga instructor, owns her own eyebrow threading business, plans on doing something in the healthcare field, and is devoted to her husband and puppy son, Mowgli. Topics in health and Hawaiian history (which is her heritage) are things that she is deeply interested in, and she hopes to share accurate and powerful information about these topics.

Emily Hyde graduated from BYU in December with a degree in Nutritional Science and a minor in International Development. During her undergraduate studies, she participated in various research opportunities, including several months working with a lab in the

Nutrition, Dietetics and Food Science Department and multiple publications of qualitative literature analysis with the Religion Department. She will be attending medical school in the fall, where she plans to be involved in global health outreach initiatives.

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